



NMHS POLICY

Falls Prevention and Management Policy

Scope (Staff)	All workers at NMHS
Scope (Area)	All NMHS sites and services
Summary	The policy outlines the minimum requirements to ensure the delivery of safe patient care through prevention and management of falls to minimise patient harms across NMHS sites and services, in accordance with the Australian Commission on Safety and Quality in Health Care (ACSQHC) National Safety and Quality Health Service Standards Comprehensive Care Standard. This policy has undergone its 3-yearly review with major amendments including: • strengthened policy principles • outlined roles and responsibilities as well as NMHS's expectation from sites and services in falls prevention and management; • strengthened the section on compliance monitoring and added requirements on reporting and evaluation of falls data to drive organisational improvement.
Approval	North Executive Team
Sponsor	Executive Director, Patient Experience and Clinical Excellence
Contact	SCGOPHCGFallsCNC@health.wa.gov.au
Version	2.0
Next Review Date	27/09/2026
NSQHS Standards	Clinical Governance (Std 1) Partnering with Consumers (Std 2) Comprehensive Care (Std 5)
ISD Number	1848
Status	CURRENT

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1. Aim

The policy outlines the minimum requirements to ensure the delivery of safe patient care through prevention and management of falls to minimise patient harms across North Metropolitan Health Service (NMHS) sites and services, in accordance with the [Standard 5: Comprehensive Care of the National Safety and Quality Health Service \(NSQHS\) Standards](#). This policy also includes consideration of risk mitigation, incident reporting and implementation of corrective actions under the [Work Health and Safety Act 2020](#) to provide a safe workplace.

Note: This policy does not apply to visitors to NMHS sites and services. If a visitor falls, first aid assistance should be offered, and person taken to the Emergency Department/Centre if required. This should be reported in accordance with the [NMHS Hazard/Incident Reporting and Investigation Policy](#) to enable follow-up and investigation. Refer to site specific procedures/guidelines for further guidance.

2. Risk

Non-compliance with this policy may result in a breach of legislative requirements, failure to meet the NSQHS Standards, harm to patients and workers, financial loss, and/or reputation and image damage to NMHS.

3. Definitions

Fall	An event which results in a person (patient, worker, member of public) coming to rest inadvertently on the ground, or other lower level. This includes: • all syncopal events; • a patient rolls out of bed; • a patient is suspected of having put themselves on the floor; • where a fall is controlled by another person; and • where a baby falls from a raised surface or a care giver's hold.
Patient	For the purpose of this policy, 'patient' refers broadly to a patient, client, people with lived experience, and any other types of consumers who attend health care at NMHS sites and services.
Worker	For the purpose of this policy and as defined by the Work Health and Safety Act 2020 : Any person who carries out work for NMHS in any capacity, including: a staff member, contractor or subcontractor, students and volunteers.

4. Governance

The NMHS Falls Management Steering Committee will be responsible for:

- oversight and endorsement of the forms/tools used for falls risk assessment and management;
- providing direction on key NMHS-wide decisions to meet the NSQHC Standards relating to preventing falls and harm from falls;
- contributing to the 3-yearly review of this policy and as required as the subject matter experts (SMEs).

4.1 Site Governance

All NMHS sites and services must have a committee within their clinical governance structure responsible for the monitoring and evaluation of falls prevention and management. The governance committee will be responsible for:

- development, endorsement and implementation of evidence-based falls prevention and management guidelines/procedures, in alignment with the principles outlined in this policy;
- collecting and reviewing falls data, and identifying trends;
- developing, implementing, and evaluating improvement strategies as required;
- escalating any risks or concerns to the local Standard 5 Comprehensive Care Committee (or equivalent);
- providing regular reports to the local Standard 5 Comprehensive Care Committee (or equivalent).

5. Policy Principles

NMHS is committed to providing a safe environment for both patients and workers across NMHS sites and services.

NMHS sites and services must develop local procedures or guidelines to provide evidence-based guidance on conducting falls risk assessment, preventing patient falls, minimising harm from falls, and post-fall management.

NMHS site and services will develop effective falls prevention programmes aiming to reduce the number of patients who fall, the rate of falls, the risk of falls and harm from falls. Falls prevention strategies should be comprehensive, multifaceted and evidence-based in order to create a safer environment and reduce harm. Innovative strategies/solutions will be encouraged. Education and training will be targeted towards workers, patients as well as the community to increase awareness of falls risk and knowledge of falls prevention and management.

All workers have an obligation under the [Work Health and Safety Act 2020](#) and the [NMHS Work Health and Safety \(WHS\) Policy](#) to maintain a safe environment. This includes identifying and reporting any hazards within the environment that may contribute to any falls in accordance with the [NMHS Hazard/Incident Reporting and Investigation Policy](#).

NMHS sites and services will have processes in place to ensure equipment used to assist with patient management after a fall (e.g., HoverMatt®) will be maintained, stored and used in accordance with manufacturer's instructions.

5.1 Risk Assessment and Falls Prevention

All clinical areas should have processes in place to ensure that a comprehensive falls risk assessment is completed and falls mitigation strategies implemented by an appropriately trained worker.

5.1.1 Emergency Department, Emergency Centre, and Maternal Fetal Assessment Unit

All patients that present at Emergency Department (ED), Emergency Centre or the Maternal Fetal Assessment Unit will have their falls risk assessed and documented during triage assessment or as soon as practicable. Falls mitigation strategies tailored to the local context will be implemented to prevent falls.

5.1.2 Inpatients

All patients should have a multifactorial risk assessment on admission or as soon as practicable after admission (within 6 hours). Assessment should be repeated:

- when transferred from one ward/department to another;

- daily or when the patient's clinical condition changes;
- after a procedure requiring sedation or anaesthetic;
- after a fall.

Workers will ensure that patients who regularly use a mobility aid have access to the most appropriate aid during their admission to NMHS. In instances where patient requires specialised mobility aids that are custom-made, the patient may bring these in for use during their admission to NMHS. Staff must take reasonable steps to ensure patient's own mobility aids are safe to use and this is to be documented in the patient's healthcare records. NMHS sites and services must outline the process details in their local guidelines or procedures.

If a mobility aid presents a risk to the safety of the worker or patient (e.g., violence and aggression risk), the mobility aid may be temporarily removed after other strategies (e.g., behavioural, environmental) have been attempted but failed. In such instance, alternative falls mitigation strategies must be implemented based on regular risk assessment for ongoing patient management. NMHS sites and services must outline process details in their local guidelines or procedures.

Workers will implement evidence-based strategies as outlined in local guidelines or procedures to prevent patient falls. These may include minimum risk reduction interventions (refer to [Appendix A](#)) as well as additional strategies for patients depending on the risk factors that require mitigation. Strategies implemented must be documented on the locally endorsed Falls Risk Assessment and Management document.

General principles for falls prevention are listed below:

- refer to the patient's mobility chart (if available) and/or care plan (or equivalent) prior to mobilising the patient;
- mobility chart (if available) and/or care plan (or equivalent) must be updated post patient fall or following a change in patient's mobility status;
- mobility should be promoted balancing the risk of falls;
- patients should be offered (where appropriate) exercise programmes to reduce and prevent deconditioning;
- all patients at risk of falls will be reviewed by a physiotherapist and occupational therapist (where services are available);
- all patients at risk of falls will have their medications reviewed by a medical officer and pharmacist (where possible);
- consider the use of other equipment to mitigate risk (e.g., an ultra-low bed, sensor alarms, hip, or head protectors);
- consider the psychosocial wellbeing for people who lose their independence; and
- worker to consider injury risk to self when assisting patients in accordance with the [NMHS Risk Management of Hazardous Manual Tasks Policy](#).

5.1.3 Community and Outpatient settings

NMHS sites and services that deliver care in the community and outpatient settings are responsible for implementing and maintaining evidence-based procedures and guidelines suitable for the clinical area to ensure patients are assessed for risk of falls and appropriate interventions are applied.

5.1.4 Patient/Family/Carer Engagement in Falls Prevention

All patients, and/or their families and/or carers will be informed about their falls risk and involved in the formulation of their falls prevention and management plan (where possible). Communication must be culturally appropriate and tailored to the health literacy, values and beliefs of the person.

Patients, their families and/or carers who speak limited or no English as well as those who are deaf have the right to access language services when using WA Health Services. This includes access to the Auslan Services. Refer to [WA Health Mandatory Policy Language Services Policy \(MP 0051/17\)](#) and any site-specific policy documents on language services.

Information related to falls prevention for patients, their families and/or carers can be located on the NMHS public website under '[Patient resources - Falls Management](#)'.

5.2 Management of Falls and Post-Falls

5.2.1 Management of a person who is falling

If a patient fall cannot be avoided, the worker who witnesses the fall will attempt, if practicable, to protect the patient's head from impact and injury. The worker must ensure, as far as reasonably possible, that they do not put themselves at risk of injury. Refer to [NMHS Risk Management of Hazardous Manual Tasks Policy](#).

5.2.2 Post-fall management

NMHS sites and services must have falls prevention and management guidelines or procedures that are suitable for the clinical environment and include process details of post-fall management based on the evidence-based principles advised by the [Post Fall Multidisciplinary Management Guidelines for Western Australian Health Care Settings 2023](#).

WA Health Multidisciplinary Post Fall Flowchart for Home Visiting Health Professionals (see [Appendix B](#)) should be used by home visiting health professionals to guide assessment and management of patients that have fallen at home within the preceding 72 hours and have not received a medical assessment. This flowchart will aid in ensuring that people who have fallen in the community receive prompt and appropriate care, which can improve outcomes and quality of life.

5.3 Clinical Handover and Documentation

Falls risk must be communicated in accordance with the [WA Health Mandatory Policy Clinical Handover Policy \(MP 0095\)](#) and any relevant local policy documents.

Falls prevention and management must be documented in the patient's healthcare record in accordance with the [NMHS Clinical Documentation Policy](#). NMHS sites and services may have additional requirements in relation to documenting patients falls.

5.4 Incident Reporting

When a patient fall occurs, it must be reported using the locally endorsed clinical incident management system (e.g., Datix Clinical Incident Management System (CIMS)) within 24 hours of the fall in accordance with the [NMHS Clinical Incident Management Policy](#).

Workers will ensure the patient, their families and/or carers are informed of the incident in accordance with the [NMHS Open Disclosure Policy](#).

If a worker is injured during the course of patient fall, it must be reported as soon as practicable to the NMHS Health Safety and Wellbeing (HSW) via the [NMHS Hazard/Incident Form](#) in accordance with the [NMHS Hazard/Incident Reporting and Investigation Policy](#).

5.5 Discharge Planning

Patients who have had previous falls as well as those who possess risk factors for falling will be at high-risk of falls after discharge. NMHS sites and services must have processes in place addressing the following minimum requirements:

- The patient's falls risk, occurrence of fall (if any) and subsequent management and outcome is documented in detail in the discharge summary in accordance with the [NMHS Discharge Summary Policy](#).
- Referral (where appropriate) to an outpatient clinic/community service provider for patients identified as a falls risk and/or for post-fall management following discharge.
- Medication review by a pharmacist and provision of education of medications prior to discharge.
- The patient, their families and/or carers are provided with information tailored to their risk factors, wellbeing and home environment, as well as the [Health advice following a fall \(WA Health\)](#).

5.6 Staff Training and Education

All sites and services are responsible for ensuring eligible workers complete falls prevention and management training as well as manual task training. At a minimum, this should include:

- common risk factors for patient falls and preventative strategies/interventions;
- guidance on how to risk assess a patient who may be at risk of falling or harm from falling using locally endorsed tools;
- guidance and training on the appropriate use of equipment for falls management;
- guidance on how to perform manual tasks safely in accordance with the [NMHS Risk Management of Hazardous Manual Tasks Policy](#);
- Guidance on how to report hazards/incident in accordance with the [NMHS Hazard/Incident Reporting and Investigation Policy](#).

NMHS sites and services are responsible for the provision of additional training requirements for falls prevention and management in specialist areas (e.g., intensive care unit, theatre, Emergency Department/Centre).

6. Compliance and Evaluation

Compliance with this policy is mandatory. The Executive Director of each NMHS site and service will be responsible for ensuring local compliance with this policy.

The relevant governing committee at each NMHS site and service will provide an annual report to the NMHS Falls Management Steering Committee on the following indicators:

- percentage (%) of falls risk assessment conducted as per this policy
- number of falls per 1000 bed days
- details of any activities undertaken to improve falls management
- results of any falls audits undertaken

NMHS Falls Management Steering Committee will submit an annual summary report on patient falls data to the NMHS Board and the North Executive Team Safety, Quality and Consumer Engagement Subcommittee.

7. Document Control

Version	Amendment(s)	TRIM Ref	Published Date
1.0	Published version		01/01/2009
1.1	Last reviewed: post fall both intrahospital and community updated		29/8/2019
2.0	Major amendment – reviewed as per three-year review process: content transferred to the new template; strengthened policy principles; outlined roles and responsibilities as well as NMHS's expectation from sites and services in falls prevention	D/23/121975	27/09/2023

	and management; strengthened the section on compliance monitoring and added requirements on reporting and evaluation of falls data to drive organisational improvement.		
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Related policies, procedures, and guidelines

[WA Health Mandatory Policy](#)

[WA Health Clinical Handover Policy \(MP 0095\)](#)

[WA Health Language Services Policy \(MP 0051/17\)](#)

[Post Fall Multidisciplinary Management Guidelines for Western Australian Health Care Settings 2023](#)

[NMHS Policy](#)

[NMHS Clinical Documentation Policy](#)

[NMHS Clinical Incident Management Policy](#)

[NMHS Discharge Summary Policy](#)

[NMHS Hazard/Incident Reporting and Investigation Policy](#)

[NMHS Occupational Safety and Health Policy](#)

[NMHS Open Disclosure Policy](#)

[NMHS Risk Management of Hazardous Manual Tasks](#)

[SCGOPHCG Policy](#)

[SCGOPHCG Clinical Handover Policy](#)

[SCGOPHCG Falls Management Guideline](#)

[WNHS Policy](#)

[WNHS Obstetrics and Gynaecology Clinical Practice Guideline Falls: Risk Assessment and Management of Patient Falls](#)

References

[Work Health and Safety Act 2020](#)

[Australian Commission on Safety and Quality in Health Care \(ACSQHC\) National Safety and Quality Health Service Standards Comprehensive Care Standard](#)

Useful resources

- [SCGOPHCG Falls Management Hub](#)
- [Stay on Your Feet/Injury Matters WA](#)
- [WA Post Fall Resources](#)
- [WA Health: Health advice following a fall](#)
- [World Guidelines for Falls Prevention & Management](#)

Appendix A. Minimum Falls Risk Reduction Interventions

Below are the minimum falls risk reduction interventions which aim to reduce the risk of falling.

Minimum interventions: As many as applicable are to be in place to mitigate the patient's risk of falling and/or injury. They should be reviewed shift by shift, on transfer to a different ward/hospital, post fall, and when there is a change of patient's medical condition.

- Provide ongoing orientation for patient to bed area, toilet facilities and ward.
- Demonstrate the use of call bell, ensure it is in reach and that they can use it effectively.
- Ensure frequently used items including mobility aids are within easy reach of patient.
- Encourage patient to use their aids such as glasses or hearing aids.
- Adjust bed and chair to appropriate height for patient.
- Minimise prolonged bed rest as it contributes to negative cardiovascular and muscle effects that may lead to falls.
- Place IV pole and all other devices/attachments on exit side of bed.
- Remove clutter and obstacles from room.
- Provide adequate lighting according to patient activities/needs.
- Encourage patient to take adequate fluids and nutrition.
- Optimise footwear where possible- discourage walking in socks/compression stockings or ill-fitting footwear. Non-slip socks are acceptable.
- Educate that all inpatients are at increased risk of falling due to injury / illness / medications.

Appendix B WA Health Multidisciplinary Post Fall Flowchart for Home Visiting Health Professionals

Patient Falls at Home

(includes witnessed or unwitnessed falls at home or in community in last 72 hours)

Assess

Reassure patient
Perform D.R.S.A.B.C.D
Administer First Aid within skill limit
Check for injury(*)
Move patient if required for safety(*)
Make comfortable until help arrives

Escalate Care?

Loss of consciousness/ Evidence of seizure
Noted change in cognition/behaviour(*)
Hit head or visible evidence Head Injury.
Signs of Injury
Pain or symptoms of clinical concern
Use of Anticoagulation medication(*)
Unable to move from floor safely or without assistance(*)

Seek Assistance

General Practitioner / Locum service
Paramedic/ Ambulance(*) CALL 000
Emergency Department
Hospital/ ED Avoidance Programs
Local Health Service Medical Officer
Anticoagulation Nurse Practitioner(*)

***See over for additional information**



Government of **Western Australia**
Department of Health

Multidisciplinary Post Fall Flowchart for Home Visiting Health Professionals

Good Practice Points

Remember

- Provide patient/ carer "**Health advice following a Fall-Community**" brochure. Review information as appropriate.
Visit: https://www2.health.wa.gov.au/Articles/F_f/Falls-prevention-and-management-in-WA
- Document fall in patients medical file(PTO for documentation prompts and criteria for completion of "CIMS form")*
- Consider referral for management of their ongoing falls risk.
Visit: <https://www.stayonyourfeet.com.au/health-professionals/referrals-to-health-professionals/> or access it directly here **SOYFWA**

Communication

- Advise your immediate supervisor/ manager of incident
- Obtain patient consent as able prior to initiating care
- Contact Next of Kin, Carer/ Care provider
Alert them to fall. Request visit I review if possible
- Contact patients GP. *Alert them to fall*
- Liaise with Multidisciplinary team
- Contact patient later in the shift or next day to check well-being.



This flowchart should be used by home visiting health professionals (HP) to guide their assessment and management of patients who have fallen at home within the preceding 72 hours *and not received medical assessment*. It is not intended to take away from clinical judgment. HP should work within their Scope of Practice when managing the patient who has fallen. If there are any concerns with acute management of a fall or ongoing management of their falls risk, they should be referred to an appropriate health service as outlined in the flow chart. For long term management of falls risk refer to the [Stay on Your Feet Health Professional](#) portal for a list of services available in WA. Refer to [WA Post Falls Guidelines 2023](#) for further information.

DATIX CIMS notification. A CIMS form should be completed when a fall occurs whilst the HP is present or the patient is on floor when the HP arrives at the patient's house. Notification of a clinical incident is made online using the Datix CIMS application or if electronic access is not available via a hard copy clinical incident form (contact your line manager). Further details re CIMS reporting can be found in the [WA DOH CIMS Policy Pack \(CIMS Policy 2019; CIMS Guidelines 2019; CIMS Toolkit 2019\)](#).

Documentation Prompts. Circumstances of fall (what, how, when), associated symptoms occurring at time of fall (e.g. dizziness, light-headedness, syncope, vertigo, weakness, incontinence, confusion / disorientation), associated falls related injuries, was the fall witnessed, environmental hazards, medical / nursing / allied health / First Aid treatment required and delivered. Include any communication performed to report the fall.

Checking for Injury. Encourage the client to stay calm and avoid moving or trying to stand whilst you undertake your assessment. Is the patient complaining of pain in the neck or head region? Can the patient move their upper and lower limbs without pain? Is there any new bruising, swelling or deformity? Has the patient's mobility changed as compared prior to the fall? Has the patient's conscious state (e.g. alertness, arousal, appropriately following commands) or cognition (e.g. memory, reasoning and orientation) changed? *Note:* Any alteration in a patient's conscious state or cognition may impact on their capacity to make decisions regarding their ongoing care including the need for further medical intervention after a fall. HP should consider further assessment if there are concerns about an acute change in the patients cognition (e.g. Delirium) using a recognised tool such as the [4AT](#) (inclusive of AMT4) or communicate with the clients GP or primary carer for further clarification regarding the patient's baseline cognitive level if there is any uncertainty.

Manual Handling - moving patient's safely. All staff are requested to follow local OSH guidelines during lifting and transfer of patient loads. Staff should consider their individual training and clinical skills prior to deciding whether to assist a patient on the floor. Staff should only consider moving a patient from the floor if:

- Risk assessment indicates that they are able to assist without injury to the patient or themselves at all times.
- Safe manual handling techniques and postures are maintained. Please refer to the local OSH Manual Handling guidelines for a description of an appropriate technique in a domiciliary setting. Staff can download the [Stay on Your Feet Up from Floor Poster \(Arms or Knees\)](#) for people able to perform the floor transfer independently, via prompting or with supervision.
- The patient needs to be removed from immediate danger.

When to call the ambulance. If a person falls at home or in the community and cannot get up independently or with minimal assistance and / or if urgent medical assistance is required an ambulance should be called (please refer to local policy and guidelines). In addition, consideration should be given to other assessment findings such as pain of clinical concern (including new neck or head pain), altered consciousness, confusion, significant injury requiring medical attention or clinical judgement suggests that there is concern over the patients wellbeing.

Communication. Staff should attempt to make contact with the patient's GP and primary carer to alert them to the fall, consequences of the falls and any interventions delivered or required. Patient consent should be obtained where possible. Staff should ensure that the patient has appropriate capacity to make an informed decision (*Refer to Checking for Injury above*) in the event that they refuse medical attention or refuse to have their GP / primary carer alerted to the fall. If the patient has capacity to make a decision, staff should abide by the patients request. However if the staff member is uncertain about the patients capacity either due to pre-existing medical conditions or as a result of the fall (e.g. hit head) then contacting the patient's GP to assist in determining capacity and the need for medical assistance may be justified if it is in the patients best interest. Staff should seek further assistance by contacting their line manager.

The fallen patient who has Coagulopathy or is on prescribed Antiplatelet and or Anticoagulant medication. Patients who are coagulopathic, on antiplatelet and or anticoagulant medications are at an increased risk of bleeding related injuries after a fall. Common medications include: Warfarin, Clexane, Rivaroxaban, Dabigatran, Apixaban, Aspirin Clopidogrel, Ticagrelor, Asasantin SR, *Dalteparin* ([A detailed list has been produced by QLD Health](#)). If the person does not require transport to Hospital, an urgent visit to their GP is recommended due to the risk of undiagnosed internal bleeding. If patient is under the management of an Anticoagulation Service contact the service provider for further advice regarding management.

DISCLAIMER: This document has been developed with care but may not be appropriate for all clinical scenarios. Clinical assessment along with medical assistance and advice must be sought if in any doubt.

Last updated April 2023

Further information can be obtained from
Falls.ManagementWA@health.wa.gov.au

This document can be made available in alternative formats on request for a person with a disability.

Within the Department of Health WA, the term Aboriginal is used in preference to Aboriginal and Torres Strait Islander, in recognition that Aboriginal people are the original inhabitants of Western Australia. No disrespect is intended to our Torres Strait Islander colleagues and community.

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